

Public Health Emergency Operations Center
« COUSP RDC »

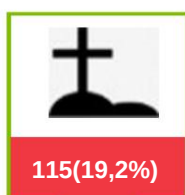
MVE-17 Incident Management System

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Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

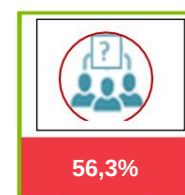
SitRep N°25/MVB_08/06/2026

Provinces affected (3)	EAST, NORTHWEST & SOUTHWEST
Affected Health Zones (25)	• Identity (17/36) : Aru, Aungba, Bambu, Bunia, Damascus, Gety, Kilo, Commander, Lita, Logo, North -Kivu (7/34) : Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha • Sud-Kivu (1/34) : Miti-Murhesa
Date the report	June 8, 2026
Publication date	June 9, 2026


 Total confirmed cases
for the 3 provinces

 Cumulative deaths
among confirmed cases
and case fatality rate

 Patients in
isolation -
hospitalisation

 Cumulative number
of recoveries

 Suspected cases of
the day

 Contact tracing rates
for the 3 provinces

*Data currently being harmonized

1. HIGHLIGHTS

- During the course of June 8, 2026, **48 new confirmed cases, including 14 deaths**, were reported in the provinces of Ituri and North Kivu. These are distributed across the health zones of **Mongbwalu (16), Bunia (11), Rwampara (11), Nyankunde (6) and Aungba (1)** in Ituri, and **Butembo (2) and Katwa (1)** in North Kivu.
- **Three (3) confirmed** Ebola virus disease patients from the Rwampara health zone (Ituri province) have been declared cured.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements toward Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

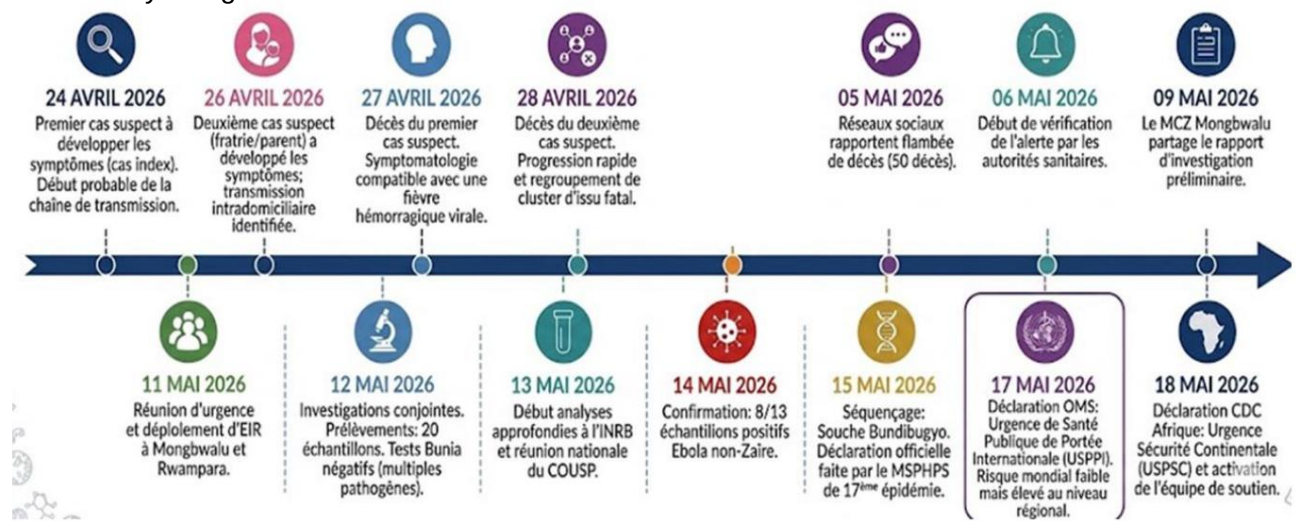


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 8, 2026

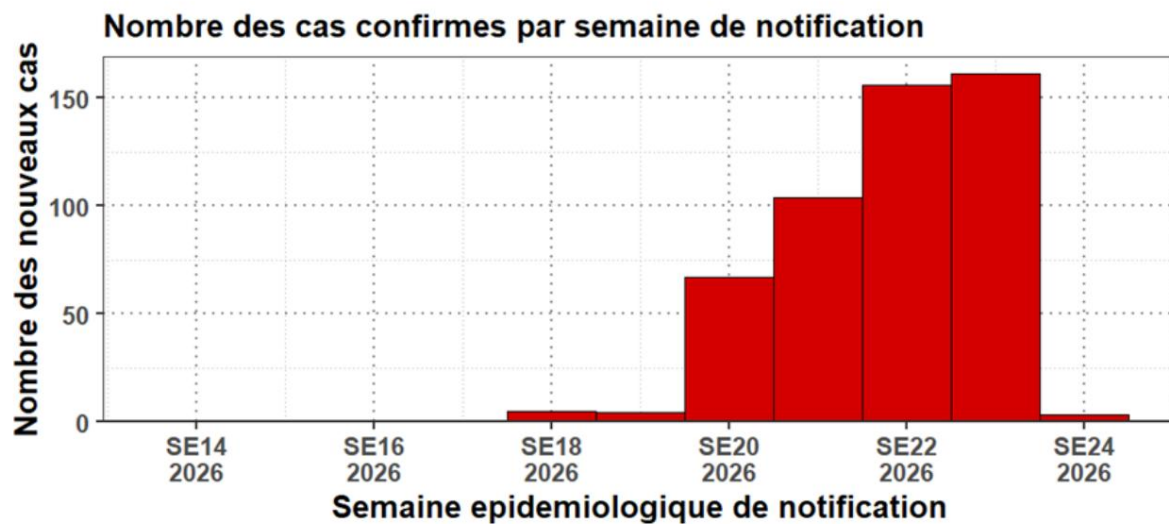
Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	563	92	16,3%	17 out of 36 (47.2%) 7	45
North Kivu	32	22	68,8%	out of 34 (20.6%) 1	3
South Kivu	3	1	33,3%	out of 34 (2.9%) 25	0
Total	598	115	19,2%	out of 104 (24.0%)	48

No new health zones were affected as of June 8, 2026. Ituri province accounts for 94.1% of the confirmed cases reported in the three affected provinces. North Kivu province remains the most affected in terms of case fatality rate (68.7%), while Ituri province maintains a low case fatality rate (16.3%). The situation remains unchanged in South Kivu province, which recorded its last confirmed case on May 26, 2026.

3.2 Distribution by health zone (Cumulative as of 08/06/2026)

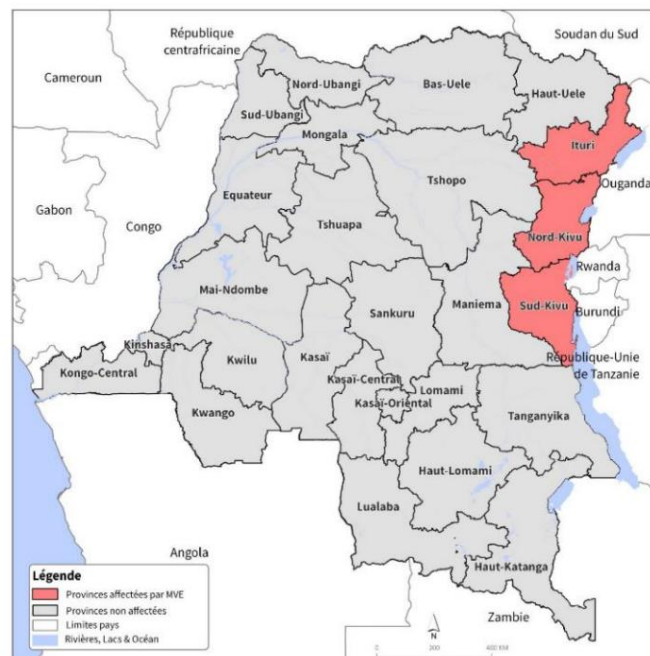
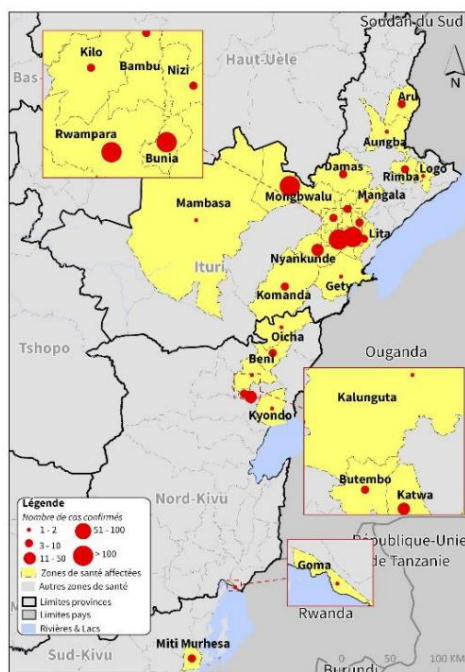
- **Ituri Province (563 cases):** Bunia (163), Rwampara (122), Mongbwalu (114), Nyankunde (32), Rimba (3), Mambasa (2), Bambu (5), Aru (3), Damascus (3), Kilo (4), Nizi (5). Ninety-four additional cases are undergoing ventilation.
- **North Kivu Province (32 cases):** Katwa (12), Beni (9), Butembo (6), Oicha (2), Kalunguta (1), Kyondo (1), Ten (1).
- **Province of South Kivu (3 cases):** Miti-Murhesa (3).

3.3 Time analysis: The number of confirmed cases is increasing week by week, indicating continued transmission of the disease within the community. A rapid geographical expansion of the epidemic is a real possibility if public health measures are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 500.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 8, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 8, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (%)
ITURI			
Bunia	163	15	9,2%
Rwampara	122	20	16,4%
Mongbwalu	114	40	35,1%
Legumes	32	1	3,1%
Bamboo	5	2	40,0%
Understand	3	1	33,3%
Kilo	4	1	25,0%
Nizi	5	0	0,0%
Mangala	1	0	0,0%
Ladies	3	0	0,0%
Aungba	2	1	50,0%
Tide	1	0	0,0%
Team	3	0	0,0%
Liter	4	0	0,0%
Logo	2	0	0,0%
Washing	2	1	50,0%
Jungle	3	0	0,0%
Other ZS (undisclosed data)	94	10	10,6%
Ituri subtotal	563	92	16,3%
NORTH KIVU			
Cut	12	8	66,7%
Me	9	7	77,8%
Butembo	6	4	66,7%
Oicha	2	2	100,0%
Kalunguta	1	1	100,0%
Kyondo	1	0	0,0%
Ten	1	0	0,0%
North Kivu subtotal	32	22	68,8%
SOUTH KIVU			
Miti-Murhesa	3	1	33,3%
South Kivu subtotal	3	1	33,3%
TOTAL	598	115	19,2%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri :

- Organization of a community dialogue between the Director General of INSP and religious leaders in the city of Bunia regarding the response to Ebola (issues, community support and public health actions implemented).
- Continued support to affected provinces under the leadership of the IM and SGI members based in Bunia, province of Ituri.
- Field visit to the Mongbwalu Health Zone under the leadership of the Director General of INSP, accompanied by the COUSP Coordinator, IM as well as technical and financial partners (OCHA, UNDP, WFP, UNICEF, WHO).
- Completion of training for the first group of 23 chief physicians of health zones on the zonal approach and AC on community surveillance.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 8, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		140	265	25	430
Alerts investigated		111	265	25	401
Investigation rate		79,3%	100,0%	100,0%	93,3%
Alerts confirmed	Alive	58	33	6	97
	Deceased	18	23	0	41
Total suspected cases for today		76	56	6	138

As of June 8, 2026, 430 alerts had been received across the three affected provinces, of which **401 (93.3%)** were being investigated. A total of **138 suspected cases were recorded** in the three affected provinces.

North Kivu province accounts for more than half (61.6%) of the notified alerts, followed by Ituri (32.5%) and the South Kivu (5.8%). In addition, the Ituri province has the lowest proportion of alerts investigated (79.3%).

Table 3. Contact tracing of confirmed cases as of June 8, 2026

Province	Contacts under follow-up	Contacts vus (n)	Follow-up rate (%)
Ituri	(n) 4,752	2 487	52,3%
North Kivu	703	533	75,8%
South Kivu	226	181	80,1%
Total	5 681	3 201	56,3%

- Continuation of field surveillance activities (reporting alerts, investigations, active search, listing and monitoring contacts, retrospective documentary review of cases and deaths, etc.).
- In-depth investigations into confirmed cases of community deaths on June 8, 2026 in the ZS.
- 478 new contacts were listed, including 326 in Nyankunde, 119 in Bunia and 33 in the Mongbwalu health zone.

- Continued support for the briefing of the 23 registered nurses in the Bunia health zone on the importance of listing and monitoring contacts through the DHIS2 tool.

Update on checkpoint and point of entry (PoC/PoE) activities

- Two PoCs were activated, one in Bey on the Komanda-Beni axis and one at Pont Ituri on the Komanda-Kisangani axis, with the support of the health zone and the community.

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) as of June 8, 2026

Indicators	Results
Proportion of activated PoCs	29,0% (16/55)
Number of travelers who passed through, % of travelers	23 467 99,2%
screened, % of travelers who washed their hands, % of travelers made aware of the risks	98,8% 82,6%
Number of alerts notified	5
Number of validated alerts	5
% of results available in 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	0
Number of contacts intercepted today	0

4.3 Laboratory

- Ituri:** 115 new samples collected and analyzed (positivity 39.1%; n=45)
- North Kivu:** 146 results remain pending due to insufficient reagents.
- South Kivu:** 6 samples received and analyzed in the laboratory, all negative for Ebola virus disease.

4.4 Infection Prevention and Control (IPC)

- Ituri:** Scorecard evaluation in 19 health centers in the health zones of Bunia (1 health center: 38.7%), Mongbwali (2 health centers: 11% and 12%), and Bambu (16 health centers: all <50.0%); start of training for 50 healthcare providers (20 supervisors and 30 focal points) from 16 health centers in the Bunia health zone on infection prevention and control (IPC); opening of 3 rings around confirmed cases in the health zones of Bunia (households, ESS and other public places identified: 10/11 sites identified, decontaminated), Rwampara (households, ESS and church identified: 8/9 sites identified decontaminated) and Mongbwali (12/17 households have been decontaminated); briefing of 19 healthcare providers (Nyakasanza Health Center, Bunia City Health Center) on waste management, hand hygiene, bio-cleaning, of 10 providers from Bora Uzima Health Center on the wearing and removal of PPE and of 5 other providers from Ndibakudu Health Center (Noah's Ark and Grace of God Health Centers) on general concepts of Ebola Virus Disease; briefing of 9 Cleaning Technicians (CTs) on Ebola Virus Disease and standard IPC precautions.
- North Kivu:** 2/4 ESS decontaminated around 2 confirmed cases in Katwa; 3 households identified and planned for decontamination in Katwa; 7/8 ESS supported and 37 providers were briefed on IPC themes in these ESS.
- South Kivu :** Decontamination of intensive care at HGR KATANA and cantonment site of Nyakadaka; evaluation score card of ESS Kalagane (40.0%) and CM Jojonika (9.7%) followed by the development of recovery plans; briefing of 12 providers of HGR Katana on decontamination, wearing and removal of PPE; evaluation of the 5 hot spots and installation of hand washing devices in the health zone of Mitimurhesa.

4.5 Holistic care

- 13 deaths recorded in CTE/CI/ESS including 9 confirmed cases (1 at CME, 1 at Rwampara: a woman 9 weeks pregnant, 4 at Mongbwalu, 1 at Elikya and 1 at Bambu) and 4 suspected cases including 2 PPL (1 at CME, 1 at Bambu, 1 at Nyankunde and 1 at Elikya).
- Five (5) children from the Saint-Nicolas orphanage, declared non-cases after 2 negative results at the CTE CME.
- 3 new recoveries at the CTE CME in the Rwampara Health Zone (Ituri province). Thus, the total number of recoveries has increased from 19 to 22, including 20 in Ituri province and 2 in North Kivu (Goma and Katwa).

Table 5. Patient movement within healthcare facilities (CTE/CT/CI) as of June 8, 2026

Indicator	Ituri	North Kivu	South Kivu	Ensemble
Patients in bed (Day -1)	286	17	7	310
Total admissions (24h)	27	7	0	34
Sorties (24h)				
Deceased (Suspects/Confessions)	13	0	0	13
No case / Cured	21	0	0	21
Escapees (Suspects/Confessions)	12	0	0	12
Transferred to the HGR	1			1
Total sorties	47	0	0	47
Patients in isolation (End of Day)	266	24	7	297
including confirmed (NC+AC) and	106	6	1	113
suspected cases	160	18	6	184

Mental and psychosocial health activities

Table 6. Key indicators of mental health and psychosocial support activities as of June 8, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
Proportion of newly confirmed cases that benefited from SMSPS interventions	3 (9,7%)	0	1(100 %)
Proportion of confirmed cases under follow-up that benefited from SMSPS interventions	30 (31,3%)	6 (100%)	5 (100 %)
Proportion of new suspected cases that benefited from SMSPS interventions	5 (18,5%)	0	
Proportion of suspected cases under follow-up that benefited from SMSPS interventions	32 (24,1%)	28(100%)	
Proportion of laboratory results having been announced	11	0	5
Including positive ones	4 (36,4%) 1	0	
Proportion of children separated in daycare who benefited from SMSPS interventions	(100,0%)	0	4(100 %)
Proportion of children separated in the community who benefited from SMSPS interventions		0	
Number of accompanying persons at the CTE who benefited from interventions SMSPS	7		10
Number of PPLs who benefited from SMSPS interventions	14	16	1

Number of households and communities that benefited from SMSPS interventions	26		
Proportion of patients who benefited from the Dignity Kits at CTE	0	0	0

4.6 Continuity of services.

- Following the evaluation of 5 health centers in Mongbwalu, several challenges and/or gaps were identified. Of the 5 centers evaluated, 3 (CECCA-20, Mongbwalu, and Abelkoko) lack adequate infrastructure, and 1 (CS plutôt) requires dispensary rehabilitation. • Finalization of recovery plans for the 3 health centers (Komanda, Mambasa, and Mongbwalu). previously evaluated.

4.7 CREC

- **North Kivu:** 13,251 people sensitized to Ebola prevention through door-to-door and mass activities; 7,589 households visited, including 3,200 with two visits and practical demonstrations (handwashing, temperature checks, safe handling), with more than 30,356 people reached; 974 Community Health Workers mobilized; 212 community actors briefed on CREC and SBC and infodemic management; 15 alerts reported (12 validated y 48h, 3 under verification) in addition to one rumor clarified with community feedback; 50 community radio stations broadcast three formats on Ebola prevention measures.
- **South Kivu:** Mass awareness campaign of 1,252 schoolchildren (637 boys and 615 girls) from KABUNGA primary school in the Kavumu health area/Mitimurhesa health zone on prevention measures and signs of Ebola virus disease; advocacy with 23 teachers and a school principal to intensify awareness in schools; briefing of 14 young U-reporter volunteers on CREC interventions around cases in the Kahungu health area/Mitimurhesa health zone affected by Ebola virus disease; 10 radio stations broadcasting prevention messages against Ebola virus disease; 640 community leaders and relays have committed to supporting public health actions in the Mitimurhesa health zone.
- **Ituri:** Organization of a community dialogue between the Director General of the National Institute of Public Health (INSP) and religious leaders in the city of Bunia; 177 people reached, including 80 (40 community actors from the Bunia Health Zone and 49 community health workers from the Rwampara Health Zone) during a briefing on Ebola prevention measures and community-based surveillance, and 97 religious leaders (including 47 women) during training on Ebola prevention measures in religious settings; 2 advocacy sessions organized with 20 people in the Bunia Health Zone (10 people: market chiefs of BIGO, Yambi Yaya, and Nyakasanza) and Nizi Health Zones (10 people: the head of the LOPA displaced persons site), respectively, focusing on their involvement in Ebola prevention activities in public places and their participation in the response (including contact tracing); 5 organized mass awareness campaigns, 385 people affected (187 M and 161 F) in 7 ZS with themes on preventive measures for Ebola virus disease, the importance of EDS, the reporting of community alerts, etc.
- **Support for other pillars :** Surveillance: Contact tracing (Mongbwalu Health Zone/Mongbwalu Health Zone); contact tracing and pre-tracing (Mudzimaria Health Zone/Bunia Health Zone). IPC: To prepare for participation in Health Decontamination Programs (Bunia and Rwampara Health Zones, Result: Health Decontamination Programs completed) and decontamination (completed).
- **Community feedback :** (i) A voicemail accompanied by a photo and messages from a doctor who died yesterday at the Ebola Medical Center (confirmed Ebola case) has gone viral, generating countless negative comments about the quality and reliability of Ebola treatment centers; (ii) several identified truck drivers belonging to a tribe frequently refuse to wash their hands at the TCHAY checkpoint and spread disease denial messages. Recommendations have been formulated for appropriate and targeted responses.

4.8 Logistics and Security

- Supply of inputs to 31 ESS/ZS and of medicines to 5 ZS (critical items).
- 135 motorcycles ensure the mobility of field teams at the level of different ZS and pillars.
- 34 vehicles (2 operational ambulances) ensure the mobility of the coordination teams. terrain in the different ZS.

4.9. Security

The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, armed groups are present in the territories of Djugu, Irumu and Mambasa . continues to restrict humanitarian access in several affected or at-risk health zones.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Start of awareness-raising activities for the PSEA pillar in the city of Goma in North Kivu.
- Briefing in Bunia of MCZ and AC of the 23/36 Health Zones of the DPS Ituri on protection against sexual exploitation, abuse and harassment and signing of the declaration of commitment.
- Briefing of 84 WFP staff and security agents (27 women and 57 men) in the Bunia health zone, 20 participants (5 women and 15 men), 95 healthcare providers (51 women and 44 men) from the Karisimbi health zone in Goma.
- Briefing of the ACoDD (the Afnet team) and PoE experts, a total of 13 participants (2 women, 11 men).
- In Miti-Murhesa in the Katana health zone, 208 participants were sensitized on PEAHS.
- PSEA sensitization to EP Kabuga more than 1000 students sensitized (South Kivu).

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3 Weakness in contact tracing (overall rate 56.3%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Supply shortage Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the 3 provinces	Delay in detection of cas	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action			Due date
Responsible pillar			
1	Training of service providers on the use of the RADIONE PCR	Laboratory	J+2
2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Surveillance/CREC D+3	
3	Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Surveillance/CREC D+3	
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri remains weak.	Surveillance	J+7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	J+2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	J+3
8	Plan for catching up on samples awaiting analysis in North Kivu Laboratory		J+2
9	North Kivu: discharge of the recovered case in Goma (Day 1), response to security incidents (ADF incursion), IPC training at Virunga General Hospital, expansion of CREC to 15,000 households		J+2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for the IT/ECZS		J+2



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